Ignazio Fiorenza — Cancer Protocol

Weekly Schedule · Updated March 2026

💊 Locked Medications

MEDICATION	ACTION	DOSE	NOTES
Ivermectin	Signal T-cells	120 mg/day (10 × 12 mg)	Take with ~50 g fat. Lipophilic — fat essential.
Fenbendazole	Disrupt cancer metabolism	444 mg/day (2 × 222 mg)	Mon/Wed/Fri nights with fat.
Mebendazole	Starve cancer cells	500 mg (1 tablet)	Tue/Thu nights with fat.

💧 On Waking — Empty Stomach (30 min before food)

Why empty stomach: Proteolytic enzymes are digested by food proteins if taken with meals. Empty stomach → systemic absorption → whole-body fibrinolytic and anti-inflammatory effects.

#	SUPPLEMENT	ACTION	NOTES
1	**Serrapeptase**	Dissolve biofilm & fibrin shield	120k–250k SPU enteric-coated. Strips tumour's fibrin cloak → exposes cancer to immune attack + HBOT oxygen.
2	**Nattovena (Nattokinase)**	Break down fibrin	Different mechanism (activates plasminogen→plasmin). Together = comprehensive shield destruction.
3	**D-Serine** NEW	Block cancer one-carbon metabolism	2–2.5 g (30 mg/kg) powder in water. Competitively blocks mitochondrial L-serine transport → depletes NADPH/GSH → cancer defenceless against Vit C/HBOT. Ramp: 1 g wk 1 → 1.5 g wk 2 → full dose wk 3.

⚠️ **Serrapeptase & Nattovena thin blood.** Stop both **2 weeks before any surgery**. Avoid with warfarin/aspirin without medical oversight. D-serine does not affect coagulation.

☀️ Morning — With Breakfast (Mon–Sat)

Take Chlorella/Spirulina **30 min before sauna** to bind mobilised toxins.

#	SUPPLEMENT	ACTION	NOTES
1	**10× Chlorella / Spirulina**	Bind & detoxify	Take before sauna. ⚠️ Stop during platinum chemo.
2	**Probiotic**	Restore gut flora	
3	**Turkey Tail**	Boost immune surveillance	3 g/day PSK extract (clinical trial dose).
4	**Astragalus** (full spectrum)	Enhance NK cells	
5	**Molybdenum** (2×/week)	Support SUOX detox	May lower copper (monitor). Take ≥2 hrs from Quercetin.
6	**ImmunoGenex**	Activate immunity	
7	**Methylene Blue**	Reset mitochondria + photosensitiser	0.5–2 mg/kg oral. Water-soluble — no fat needed. 30 min before Red Light → photodynamic kill (665 nm).
8	**Astaxanthin**	Guard cell membranes	12–24 mg/day with fat. Promotes Akkermansia growth.
9	**Pasteurised Akkermansia**	Rebuild gut barrier	10B CFU/day with food. Astaxanthin amplifies colonisation.
10	**L. gasseri Yoghurt**	Repopulate microbiome	½ cup (120 mL). Ramp from ¼ cup in week 1.

🔬 Daily Therapy Sequence (Mon–Sat)

Order matters — each step primes the next.

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MB with breakfast → 30 min → Red Light → Resistance Training → HBOT → Sauna → Shower → Spooky2
                (2–3×/wk) · · · · · · · · · · (5×/wk) · · · · · · · · · · · · · · · · · (daily)
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STEP	THERAPY	WHY THIS ORDER	DURATION
1	**Methylene Blue**	MB needs 30 min to reach blood before Red Light activates it.	—
2	**Red Light Therapy**	660+850 nm activates MB (photodynamic kill). Primes mitochondria for HBOT.	10–20 min
3	**Resistance Training**	Activate myokines. Train before sauna (heat amplifies recovery).	20–30 min, 2–3×/wk
4	**HBOT** (Hyperbaric Oxygen Therapy)	Flood exposed tumour with O_2 (Serrapeptase stripped fibrin shield). Hospital-grade, 100% O_2.	30–60 min at 3 ATA
5	**Far-Infrared Sauna**	Deep heat detox. Mobilises toxins Chlorella binds. Synergy with Ivermectin.	30 min at 54–60°C
6	**Shower**	Wash off mobilised toxins.	—
7	**Bio-Resonance — Spooky2**	Shatter cancer cells with targeted frequencies. Can run during rest/work.	30–60 min (Plasma/Contact)

⚠️ **2–3 hour gap** between sauna and night medications. Eye protection for Red Light. Do NOT apply Red Light over tumour without MB.
Spooky2: Phase 1 = Terrain detox (10–15 days). Phase 2 = Cancer frequency sets. Avoid during chemo. Hydrate well.

🍽️ Lunch — Polyphenol Synergy Cluster (Mon–Sat)

Why together: Quercetin inhibits COMT → ↑ EGCG levels. Vit C stabilises EGCG. Anthogenol OPCs amplify EGCG + Quercetin. Resveratrol + Quercetin share SIRT1/AMPK pathways. 5 polyphenols = **multi-target metabolic siege**.

#	SUPPLEMENT	ACTION	NOTES
1	**Green Tea (EGCG)**	Inhibit cancer growth	Take at start of meal (before main food). Avoid dairy (casein blocks EGCG). ≤800 mg/day. Monitor liver enzymes.
2	**Vitamin C + D3/K2**	Pro-oxidant kill + stabilise EGCG	Oral liposomal Vit C (3–10 g/day) on non-IV days.
3	**Quercetin**	Block cancer metabolism	With some fat. ⚠️ CYP3A4/CYP2C19 inhibitor.
4	**Resveratrol**	Suppress tumour pathways	★ MOVED from morning. ≤1 g/day. Complements Quercetin.
5	**Anthogenol** (grape seed)	Anti-angiogenic OPCs	★ MOVED from morning. Amplifies EGCG + Quercetin.
6	**10× Chlorella/Spirulina**	Bind & detoxify	2nd dose.
7	**Zinc 15 mg**	Restore apoptosis	Zinc-L-Carnosine for colon. Synergy with Butyrate.
8	**Butyrate**	Feed colon lining cells	Supports peristalsis. PD-L1 reduction complements Ivermectin.

🌙 Night — Take with Fat

Mon / Wed / Fri

#	SUPPLEMENT	ACTION	NOTES
1	**Fenbendazole + Ivermectin**	Disrupt & signal	With ~50 g fat. Two-front war.
2	**Turkey Tail**	Boost immune	Stacks with Ivermectin ICD.
3	**10× Chlorella/Spirulina**	Bind & detoxify	3rd dose.
4	**Butyrate**	Feed colon lining	Synergy with Marshmallow.
5	**Marshmallow**	Soothe + prebiotic	Feeds butyrate bacteria.
6	**Magnesium Glycinate**	Enzyme repair	200–400 mg. Promotes sleep.
7	**Melatonin 3 mg**	Suppress tumour	⚠️ Trials use 20 mg.

Tue / Thu / Sat

#	SUPPLEMENT	ACTION	NOTES
1	**Mebendazole** (not Sat) **+ Ivermectin**	Starve & signal	With ~50 g fat.
2	**Turkey Tail**	Boost immune	2nd dose.
3	**10× Chlorella/Spirulina**	Bind & detoxify	3rd dose.
4	**Butyrate**	Feed colon lining	
5	**Marshmallow**	Soothe + prebiotic	
6	**Magnesium Glycinate**	Enzyme repair	200–400 mg.
7	**Melatonin 3 mg**	Suppress tumour	⚠️ Trials use 20 mg.

Sunday is a rest day — no supplements scheduled.

💉 Injections

#	INJECTION	ACTION	DOSE	FREQUENCY	NOTES
1	**IV Vitamin C**	Pro-oxidant flood	25–75 g (1–1.5 g/kg)	Fortnightly (Day 1)	G6PD test MANDATORY. Synergy with HBOT.
2	**IV Glutathione**	Detox & protect organs	600–2,000 mg	Fortnightly (Day 2)	≥24 hrs AFTER Vit C. Neutralises pro-oxidant kill if same day.
3	**Methylcobalamin (B12)**	DNA repair + nerve protection	1–2 mg IM injection	Weekly (then monthly)	Active form — no liver conversion needed. Supports methylation + RBC production.

🩸 Blood Test Schedule

Baseline (One-Time, Before Starting)

TEST	WHY
G6PD screen	Clears you for IV Vit C + Methylene Blue
Full Blood Count (FBC)	Red/white cells, platelets baseline
Liver Function (LFTs)	ALT, AST, GGT — baseline before EGCG + drug load
Kidney Function (U&E)	Creatinine, eGFR — baseline before IV Vit C
CEA (tumour marker)	Your tracking number for colon cancer
CRP	Inflammation marker — tracks overall progress
Copper + Zinc levels	Baseline before Molybdenum + Zinc supplementation
Vitamin D	Baseline for D3/K2 dosing (target 60–80 ng/mL)
RBC Magnesium	Serum Mg is unreliable — RBC test is accurate
Coagulation (INR/PT)	Baseline before Serrapeptase + Nattovena
B12 + Folate	Baseline before Methylcobalamin injections

Every 6 Weeks (Ongoing)

TEST	WHAT YOU'RE WATCHING
CEA	↓ Trending down = protocol working. ↑ Rising = reassess.
CRP	↓ Dropping = inflammation reducing. ↑ Rising = investigate.
FBC	Platelets OK? (Serrapeptase/Nattovena). Red cells OK? (drug load)
LFTs	Liver coping with EGCG + Ivermectin + FBZ + MBZ?
Kidney Function	Handling the supplement + IV load?
Copper	Molybdenum may lower it (monitor — low copper is anti-angiogenic)
Vitamin D	Maintaining therapeutic range?

> **The two key numbers: CEA** (tumour marker) and **CRP** (inflammation). If both are trending down, the protocol is working.

🔗 Synergy Map

SYNERGY	MECHANISM	TIMING
D-serine → IV Vit C / HBOT	D-serine depletes NADPH/GSH via 1C metabolism block → cancer defenceless	D-serine waking, Vit C/HBOT ≥3 hrs later
D-serine + FBZ/MBZ	Serine blockade + glucose blockade = dual metabolic starvation	D-serine morning, FBZ/MBZ night
D-serine + EGCG	1C input blocked (D-serine) + folate output blocked (EGCG)	D-serine morning, EGCG lunch
D-serine + MB + RLT	GSH depletion → $^1O_2$ not quenched → amplified photodynamic kill	D-serine waking, MB breakfast, RLT +30 min
MB → Red Light	MB absorbs 665 nm → singlet oxygen → photodynamic kill	MB 30 min before RLT
Serrapeptase → HBOT	Strip fibrin shield → O_2 floods exposed tumour	Waking → mid-morning
RLT → HBOT	Red light primes mitochondria → HBOT maximises ROS	RLT before HBOT
HBOT + Melatonin	Combo studied in colorectal cancer — inhibitory	Melatonin at night
Chlorella → Sauna	Chlorella binds toxins mobilised by sweating	Chlorella 30 min before
EGCG + Vit C	Vitamin C stabilises EGCG → better absorption	Same meal (lunch)
Quercetin + EGCG	Quercetin inhibits COMT → ↑ intracellular EGCG	Same meal (lunch)
Anthogenol + polyphenols	OPCs amplify EGCG + Quercetin + Resveratrol	Same meal (lunch)
Resveratrol + Quercetin	Both activate SIRT1/AMPK; complementary epigenetics	Same meal (lunch)
Zinc + Butyrate	Zinc heals mucosa + Butyrate feeds colon lining	Same meal (lunch)
Butyrate + Marshmallow	Marshmallow feeds butyrate-producing bacteria	Same meal (night)
Astaxanthin + Akkermansia	Astaxanthin promotes Akkermansia colonisation	Same meal (morning)
Ivermectin + Butyrate	IVM triggers ICD; Butyrate reduces PD-L1	Both at night
Fenbendazole + Ivermectin	Metabolic starvation + immune activation	Same meal (night)
Serrapeptase + Nattovena	Different fibrinolytic mechanisms = synergy	Together (on waking)
Nattovena → Vit C IV	Improved microcirculation → Vit C pro-oxidant H_2O_2 reaches tumour better	On waking → IV day
Molybdenum + Butyrate	SUOX clears sulfite → supports butyrate metabolism in colon	Morning + lunch/night
Sauna + Ivermectin	Published hyperthermia + IVM synergy	Sauna AM, IVM PM

❌ Hard Contraindications

INTERACTION	RISK
Methylene Blue + SSRIs	Serotonin syndrome — potentially fatal.
Methylene Blue + G6PD deficiency	Haemolytic anaemia. G6PD test mandatory.
IV Vitamin C + G6PD deficiency	Haemolytic anaemia. G6PD test mandatory.
Red Light over tumour (without MB)	May stimulate tumour growth. Only with MB photosensitiser.

⚠️ Timing Separations

PAIR	RULE	WHY
D-serine ↔ Oral Vit C	D-serine ≥3 hrs before Vit C	Let NADPH depletion establish before pro-oxidant strike
D-serine ↔ Food	Take on empty stomach with Serrapeptase	Avoid L-serine competition from dietary amino acids
Serrapeptase / Nattovena	Empty stomach — 30 min before food	With food they digest proteins instead of absorbing
Molybdenum ↔ Quercetin	Different meals (AM vs lunch)	Quercetin inhibits xanthine oxidase (Mo-dependent)
Zinc ↔ Magnesium	6+ hrs apart (lunch vs night)	Compete for absorption at high doses
IV Vit C ↔ IV Glutathione	≥24 hrs apart	Glutathione neutralises Vit C's pro-oxidant kill
IV Vit C ↔ Chemo	24–48 hrs	Coordinate with oncologist
Sauna ↔ Night meds	2–3 hr gap	Sweating may reduce drug absorption
EGCG	Start of meal, before main food	Avoid dairy (casein blocks EGCG)

⚠️ Monitoring Required

ITEM	MONITOR
Serrapeptase + Nattovena	Blood-thinning — stop 2 wks pre-surgery. Avoid with anticoagulants.
Anthogenol	Enhances anticoagulant effects. Monitor on blood thinners.
Quercetin + Resveratrol	Both inhibit CYP3A4/CYP2C19 → may ↑ drug levels.
Chlorella + Platinum chemo	Chelation may bind cisplatin. **Stop during platinum chemo.**
EGCG ≥600 mg/day	Monitor liver enzymes.
Molybdenum	May lower copper — beneficial but monitor.
Ivermectin 120 mg/day	Above standard dose. Fat essential. Medical supervision required.
Melatonin	Protocol: 3 mg. Cancer trials: **20 mg/day**. Discuss escalation.
Serrapeptase + Drugs	May ↑ blood levels of co-administered drugs.
Resveratrol >1 g/day	GI discomfort; CYP3A4 effects increase.
D-serine	Renally excreted — monitor creatinine + eGFR at each blood draw. If eGFR < 60, reduce or stop. Ramp: 1 g wk 1 → 2.5 g wk 3.